

Definitive Chemoradiotherapy for Esophageal Carcinoma

S. Lewis Cooper, MD^a, J. Kyle Russo, MD^a, Steve Chin, MD^{b,c,*}

KEYWORDS

- Radiotherapy • Esophagus • Esophageal • Carcinoma • Definitive
- Chemoradiotherapy

KEY POINTS

- The management of esophageal cancer is complex and should be managed in a multidisciplinary setting including thoracic surgery, medical oncology, radiation oncology, diagnostic radiology, pathology, gastroenterology, and nutrition.
- Superficial esophageal cancer (cT1N0) is best managed by endoscopic therapy (endoscopic mucosal dissection or endoscopic mucosal resection) or esophagectomy, but definitive radiotherapy may benefit a small population of patients with squamous cell carcinoma and who are not candidates for other therapies.
- Locally advanced esophageal cancer is best treated with neoadjuvant chemoradiotherapy (CRT) followed by esophagectomy, but areas of future research include identifying patients who may not benefit from esophagectomy after CRT.
- Patients who have unresectable disease, are not operative candidates, or refuse surgery should receive definitive CRT and can be expected to have a 2-year survival of 40% to 55%.
- Patients with a radiographic complete response on positron emission tomography can have a 2-year survival as high as 70%.

INTRODUCTION

Although some modest survival gains in esophageal carcinoma have been made over the past 4 decades, survival remains poor. Globally, esophageal cancer is the sixth leading cause of cancer mortality, with an estimated 406,800 deaths and 482,300 new cases in 2008.¹ Esophageal cancer will affect about 17,460 people in the United States in 2012 and an estimated 15,070 people will die of the disease.² The 5-year survival has improved from 5% in 1977 to 19% in 2007. Gains have been made in

The authors have nothing to disclose.

^a Department of Radiation Oncology, Medical University of South Carolina, Charleston, SC, USA; ^b Neuropsych-Oncology, Department Psychiatry and Behavioral Sciences, Medical University of South Carolina, Charleston, SC, USA; ^c Division of Hematology and Oncology, Medical University of South Carolina, Charleston, SC, USA

* Corresponding author. 96 Jonathan Lucas Street-903 CSB, Charleston, SC 29425.

E-mail address: chin@muscc.edu

Surg Clin N Am 92 (2012) 1213–1248

<http://dx.doi.org/10.1016/j.suc.2012.07.013>

0039-6109/12/\$ – see front matter Published by Elsevier Inc.

surgical.theclinics.com